

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

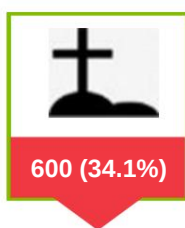
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°054/MVB_07/07/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (37)	- Ituri (25/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa and Boga. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	July 7, 2026
Publication date	July 8, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 2 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- In the past 24 hours, **51 new confirmed cases and 20 deaths (55% community) have been recorded** in the provinces of Ituri (45 cases and 20 deaths) and North Kivu (6 cases) highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo. Thus, the cumulative total stands at 1,759 confirmed cases and 600 deaths, expressing an overall case fatality rate of 34.1%.
- Nine (9)** deaths were recorded among patients with Ebola virus disease in Ebola treatment centers in the Ituri provinces (4 at CT Nizi, 2 at CTE Bunia, 1 at CTE Mongbwalu, 1 at CTE ISTM and 1 at CTE Rwampara).
- Five (5)** patients with Ebola virus disease in Ituri province (2 at CT SOTA, 1 at CTE Rwampara, 1 at CTE Mongbwalu and 1 at CTE Nyakunde) have been declared cured after obtaining negative control tests.
- Two (2)** cases have been confirmed in **Kisangani**, Tshopo province. One of them is linked to the Nia-Nia health zone, and the other appears to have no geographical connection outside of Kisangani. The cases are positive PCR testing using RADIONE and RT-PCR validation tests (Altona) are underway, along with in-depth investigations. Operations are also in progress in Kisangani to establish and strengthen all pillars of the response. These two cases are not currently included in the total number of cases presented in this status report, and will be included in future status reports, after validation.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has been facing a crisis

Chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million inhabitants, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, the Ituri province had already been affected by the 10th Ebola virus (EV) epidemic, which was then raging in North Kivu. The current epidemic, caused by Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total population estimated at nearly 15 million inhabitants, with massive internal displacements and cross-border flows to neighboring countries.

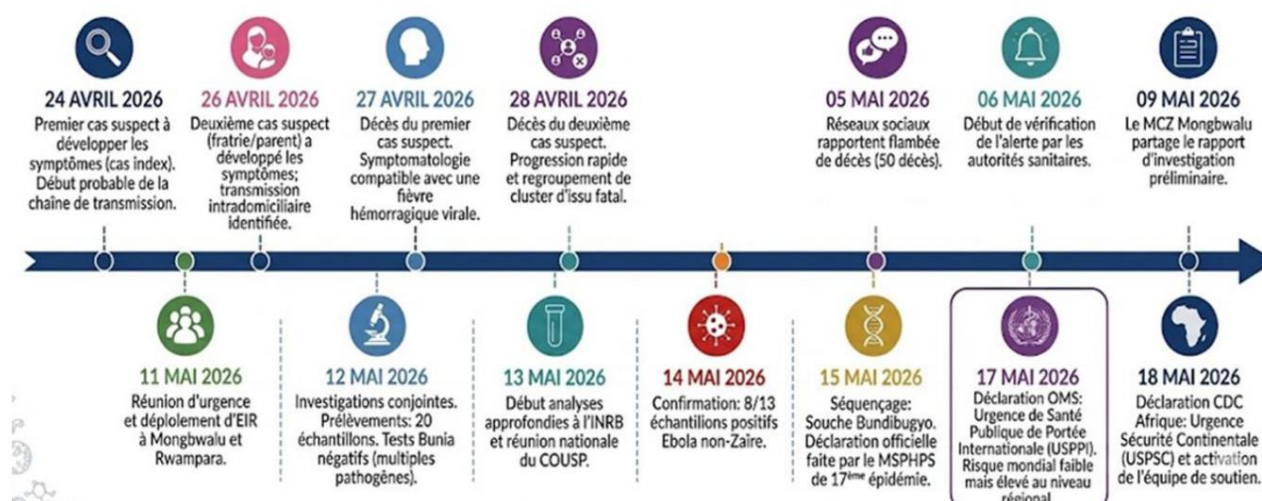


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of July 7, 2026

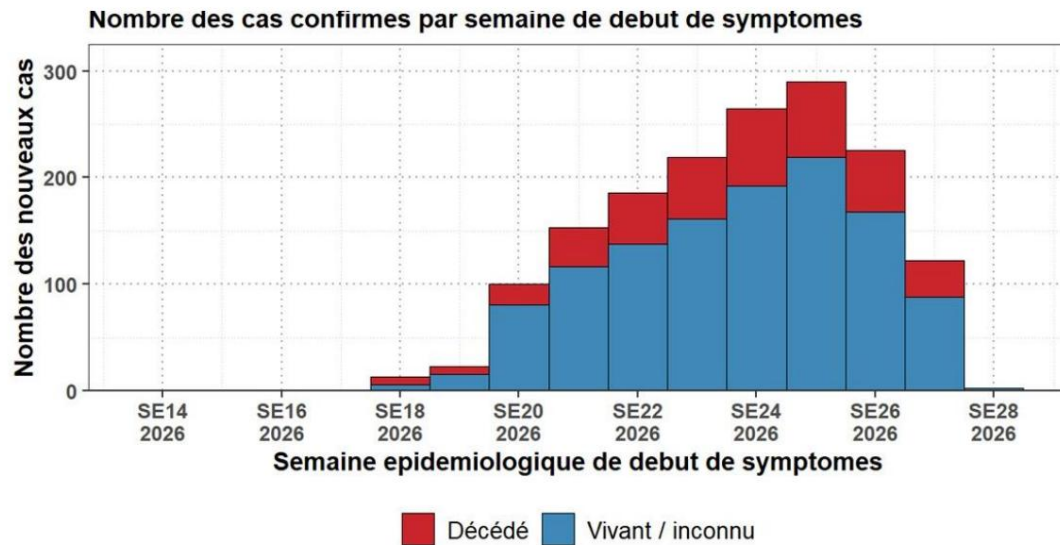
Table 1. Distribution of confirmed cases and deaths by affected province as of July 7, 2026.

Province	Confirmed cases	Death (confirmed)	Lethality	Affected health zones	New cases confirmed (24h)
Ituri	1601	511*	31.9%	25 out of 36 (69.4%)	45
North Kivu	155	88*	56.8%	11 out of 34 (32.4%)	6
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	1,759	600	34.1%	37 out of 104 (35.6%)	51

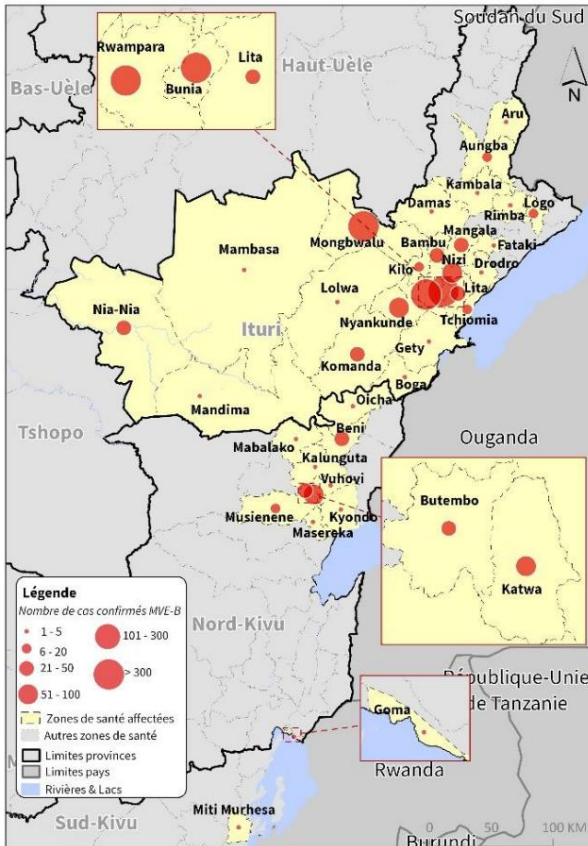
*9 deaths in Ituri were recorded among patients with Ebola virus disease (previous confirmed cases) in Ebola treatment centers/treatment centers.

At the end of the day on July 7, 2026, a new health zone was affected in Ituri. The number of affected health zones rose to 37 out of 104 (section 3.1). Since May 26, 2026, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission.

3.2 Time-bound analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capacities) confirmed continued and increased community transmission in week 25. Compared to all weeks in which cases have been recorded since the beginning of the epidemic, weeks 25 and 26 continue to show a growth in reported cases, with more than 300 cases per week.



3.3 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of July 7, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (July 7, 2026)

Province / Health Zone	Cumulative confirmed cases	Cumulative confirmed deaths (n)	Case fatality rate (CFR%)
ITURI	1601	511	31.9%
Bunia	483	137	28.4%
Rwampara	364	76	20.9%
Mongbwalu	310	165	53.2%
Nyankunde	97	20	20.6%
Nizi	94	28	29.8%
Lita	40	16	40.0%
Komanda	23	8	34.8%
Bamboo	28	9	32.1%
Kilo			11.1%
Mangala	9	1	50.0%
Tchomia	40 15		13.3%
Damascus	5		0.0%
Aungba	6		33.3%
Rimba	4		0.0%
Aru	3		33.3%
Mambasa	4		75.0%
Logo	7		0.0%
Drodro	3		100.0%
Gety	1		0.0%
Kambala	2		100.0%
Nia-Nia	35	20	37.1%
Fataki	4		25.0%
Mandima	5	2 0 2 0 1 3 0 3	40.0%
Lolwa			100.0%
Boga			100.0%
Other areas not yet identified	1 1 17	1 1 0	0.0
NORTH KIVU	155	88	56.8%
Butembo	42	20	47.6%
Katwa	61	41	67.2%
Blessed	29	19	65.5%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	4	1	50.0%
Goma		2	0.0%
Masereka	1	0	0.0%
Vuhovi			100.0%
Mabalako		0	0.0%
Musienene	3	1	25.0%
SOUTH KIVU	1 1 8 3	0 2 1	33.3
Miti-Murhesa	3	1	33.3
TOTAL	1759	600	34.1%

Over the past 24 hours, **51 new confirmed cases and 20 deaths (55% community) have been recorded** in the provinces of Ituri (45 cases and 20 deaths) and North Kivu (6 cases), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo. Thus, the cumulative total stands at 1,759 confirmed cases and 600 deaths, expressing an overall case fatality rate of 34.1%.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **91.0%** of cases (n=1,601) and **85.2%** of deaths (n=511), representing a crude case fatality rate of **31.9%**, making it the epicenter of the epidemic. The health zones of Bunia (483 cases), Rwampara (364 cases), Mongbwalu (310 cases), Nyankunde (97 cases), and Nizi (94) remain the most affected, accounting for approximately 84.2% of reported cases at the provincial level and 76.6% at the national level. In Nizi, the epidemic currently progressing faster than in other areas.

Severity of the epidemic in North Kivu

A total of **155 confirmed cases and 88 deaths** have been reported in North Kivu province since the start of the outbreak, representing a **crude case fatality rate of 56.8%**. This high case fatality rate is concentrated in the health zones of **Katwa** (67.2%), **Oicha** (66.7%), **Beni** (65.5%), **Kalunguta**, and **Kyondo** (50.0%), respectively. The Katwa health zone has the highest number of reported cases (n=61), followed by the Butembo health zone (n=42) with a case fatality rate of 47.6%. A case fatality rate of 100% was observed in the exceptionally small Vuhovi health zone.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri & Kinshasa

- Holding the coordination meeting with the response team and technical partners and financial support for field activities.

National level :

- Coordination meeting with an agenda: 100%
- Action items completed for daily coordination meetings: 80% **Provincial level :**
- Coordination meeting with an agenda: 100% for ITURI and North Kivu; South Kivu: ND
- Action items completed for daily coordination meetings: N/A for the three provinces (ITURI, North Kivu and South Kivu)

Zonal Level : •

- Coordination meeting with an agenda: N/A
- Action items completed for daily coordination meetings: N/A

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of July 7, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Report (D-1)			0	49
New Alerts		0	9	1,171
Total alerts for today		759	9	1,220
Alerts investigated		759	9	970
Investigation rate		759	100.0% 3	79.5%
Alerts confirmed	Alive		0	212
	Deceased	49,403,452,202 44.7% 180,600 71 28		92

Total suspected cases for today	202	99	3	304
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During the day of July 6, 2026, a total of **1232 alerts** were recorded, including **1171** new alerts, demonstrating the continued active epidemiological surveillance in the affected provinces.

The majority of alerts were reported in North Kivu (**759, 64.8%**), followed by Ituri (**403, 34.4%**) and South Kivu (**9, 0.8%**).

Of the alerts recorded that day, **1232 (78.7%)** were investigated. The investigation rate reached 100% in North Kivu and South Kivu, while it stood at **43.5%** in Ituri, highlighting a need to strengthen investigative capacities in that province.

The investigations led to the validation of 306 suspected cases, of which **214 (69.9%)** involved living individuals and **92 (30.1%)** community deaths. Ituri recorded the highest number of validated suspected cases with **202 (65.4%)**, followed by North Kivu with **99 (32.4%)** and South Kivu with **3 (0.9%)**.

Continued strengthening of surveillance, rapid investigation of alerts and early detection of cases, particularly in Ituri, remains essential to interrupt the chains of transmission.

Table 4. Contact tracing of confirmed cases (1) as of July 7, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri*	11,788	9,309	78.9%
North Kivu	2,331	1,991	85.4%
Total	14,119	11,300	80.0%

* In Ituri, the follow-up rate was calculated only for health zones that had submitted reports (completeness: 78.1%). The health zones of Lita, Damas, Rimba, Kambala, Logo, Fataki, and Mandima did not have data available at the time of report consolidation.

As of July 7th, **14,119** contacts were under surveillance in the provinces of Ituri and North Kivu. Of these, 11,300 were actually seen, corresponding to an overall follow-up rate of **80.0%**.

Ituri had the highest number of contacts under follow-up, 11,788 (83.5% of the total), with 9,309 contacts seen, representing a follow-up rate of 78.9%. In North Kivu, 2,331 contacts were under follow-up, of which 1,991 (85.4%) were visited.

Contact tracing performance is comparable between the two provinces; however, the overall follow-up rate remains below the recommended operational threshold for optimal contact monitoring. Strengthening daily contact tracing, particularly of those who have not been seen, remains a priority to ensure the early detection of secondary cases and limit the risk of community transmission.

Table 5. Contact tracing of confirmed cases (2) as of July 7, 2026

Province	Number of new contacts		Number of contacts had become suspects		Number of contacts outings every 21 days	
	n		n		n	%
Ituri	571	88.8 %		100.0 %	214	40.4%
North Kivu	72	11.2%		0.0%	316	59.6%
Total	643	100.0%	404	100.0%	530	100.0%

The day ended on July 7th with **643 new contacts** listed, the majority in Ituri (571, 88.8%), while North Kivu listed 72 (11.2%). Among the contacts under monitoring, four (4) have developed symptoms

compatible with the definition of suspected case, including four (0.03%) in Ituri, requiring immediate investigation and management in accordance with current procedures.

Furthermore, 530 contacts successfully completed their 21-day follow-up period without developing symptoms consistent with the disease in North Kivu.

These results demonstrate the continuation of contact tracing and follow-up activities, while highlighting the need to maintain rigorous surveillance to ensure early detection of secondary cases and to quickly interrupt any potential chain of transmission.

Update on checkpoint and point of entry (PoC/PoE) activities

- **Ituri** : Four (4) alerts were notified to the PoE/PoC, including two (2) live alerts and two (2) deceased individuals. The live alerts were confirmed, isolated, and referred to the transit center. The two deceased individuals were swabed.
- **North Kivu**: Three (3) alerts were notified to the PoE/PoC, including one (1) live alert at PoC Kahongo, invalidated after secondary screening, and two (2) lifeless bodies intercepted at PoC Mukulia then referred to the morgue of the HGR of Beni for sampling.

Table 6. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of July 7, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	98.2% (54/55)	100.0% (14/14)	98.6% (68/69)
Number of people who went through PoE/PoC testing,	96,657	59,687	156,344
% of travelers screened, % of	91.3%	97.9%	93.8%
travelers who washed their hands, % of	93.8%	97.9%	95.3%
travelers made aware of the issue	99.6%	97.9%	99.0%
Number of alerts notified	4	3	7
Number of validated live alerts	2	0	2
Number of lifeless bodies intercepted today	2	2	4
Number of problematic contacts intercepted today; %			0
of validated live alerts referred to CT/CTE; % of	0 100.0%	0	100.0%
deceased bodies swabed	100.0%	0.0%	100.0%
Number of positive results of referred suspected cases	0	100.0% 0	0

4.3. Laboratory

- **Ituri** : 147 samples collected and analyzed (**positivity 30.6%; n=45**).
- **North Kivu** : 90 samples collected and analyzed (**positivity 6.7%; n=6**).
- **South Kivu** : 3 samples collected and ongoing analysis.

4.4. Infection Prevention and Control (IPC)

Ituri: 21 households were identified around the cases and decontaminated. One identified healthcare facility was also decontaminated. Twenty-nine (29) deaths were recorded for DSS activities, of which 22 DSS tests were carried out, representing a completion rate of 76%, and 18 post-mortem samples were taken.

North Kivu: Scorecard evaluations carried out for 10 ESS in the health zones of Beni (5 ESS), Mutwanga (2 ESS), Oicha (3 ESS) whose average score is 26.1% and max score 37.1%, no ESS reached 50%;

57 ESS monitored and supported in the Beni (20); Kyondo (1); Oïcha (20), Butembo (12) health zones and Musienene (4 (4) were secured; Provision of PCI KITs in 8 ESS in Beni by IMC (CS BOIKENE, CS BUTSILI, BUNDJI, CM JADES, CS TUUNGANE, CS MABAKANGA, CM GRACIA AND CH CARLE BECKER) 155 service providers briefed, including 18 in Musienene, 4 in Kyondo, 57 in Butembo and 76 in Oïcha; Briefing launched PCI WASH of 19 Community and religious leaders in the Butembo health zone on barrier measures MVE

4.5 Holistic care

Ituri:

- **Nine (9) deaths were recorded among patients with Ebola virus disease in the care facilities of Ituri: five (5) confirmed deaths in the Ebola Treatment Centers and four (4) confirmed deaths in the transit center.**
- **Five (5) patients with Ebola virus disease have been declared recovered after obtaining negative follow-up tests**
- By the end of the day on July 7th, 927 hot meals had been served, including 131 to suspected cases, 146 of the confirmed cases, 601 to PPLs and 49 to accompanying persons.

Patient movement:

- Seventy-one (71) new admissions were recorded in Ituri, including four (4) PPLs and sixty-seven (67) other patients.
- No transfers to a general referral hospital have been reported.

Table 7. Patient movement within healthcare facilities (CTE/CT/CI) as of July 7, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	505	165	24	694
Total admissions (24h)	71	32	15	118
Exits (24h)				
Deceased (Suspects/Confessions)	16	0	0	16
No case	28	4	0	32
Healed	5	0	0	5
Escapees (Suspects/Confessions)	9	0	0	9
Transferred to the HGR	0	0	0	0
Total outflows	58	4	0	62
Patients in isolation (End of Day),	518	193	39	750
including confirmed cases	212	20	0	232
(NC+AC) and suspected cases	306	173	39	518
Overall occupancy rate	85.0%	137.0%	86.6%	94%

In total, 694 patients were in isolation, including 232 confirmed cases and 518 suspected cases, for an overall bed occupancy rate of **94%**.

Mental health activities and psychosocial support

Table 8. Key indicators of mental health and psychosocial support activities as of July 7, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	5 (41.7%)	0	0
Confirmed cases currently being monitored that have received interventions SMSPS	61 (41.8%)	7	0
New suspected cases that benefited from SMSPS interventions	8 (22.9%)	3	1
Suspected cases under follow-up that have received interventions SMSPS	70 (52.2%)	5	3 (100.0%)
Laboratory results have been announced.	7	4	0
Including positive ones	6	0	0
Number of children separated in daycare who benefited from interventions SMSPS	1	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	0	5
Number of accompanying persons at the CTE who benefited from interventions SMSPS	38	2	0
Number of PPLs who benefited from SMSPS interventions	28	2	0
Number of household and community members who benefited from SMSPS interventions	31	0	0
Number of EDS supported	6	0	0

4.6 Continuity of care

- Closing of the health training for 20 midwives and nurses from the Bambu health zone sexual and reproductive health in the context of Ebola virus disease in Ituri.
- In total, **112 PPLs** are confirmed, of which **36 have recovered**, 35 have died (31.2%), and 21 are hospitalized in Ituri province.

4.7 Risk communication and community engagement

Ituri: Thirteen (13) people were engaged during an advocacy meeting (8 men and 5 women); 1,113 people were reached during home visits (485 men and 628 women); 826 people were reached during educational talks and focus groups (324 men and 502 women); 52 people participated in a community dialogue (41 men and 11 women); 4,878 people were reached during 15 mass awareness campaigns (2,076 men and 2,802 women); and 6,080 people were reached in churches, mosques and markets (2,978 men and 3,102 women).

Community feedback: 13 feedbacks and infodemics were captured, of which 10 were clarified, i.e. 76.9%.

A major concern was the return of some patients home due to the high volume of patients in the CTEs.

Support for the pillars of the response

Surveillance: 82 community alerts were reported, including 49 live alerts and 33 deaths, in the health zones of Bunia, Nyakunde and Nia-Nia.

CREC: 18 religious leaders (16 men and 2 women) were briefed in the Bunia health zone, and 30 community health workers (22 men and 8 women) were briefed in the Aungba health zone. Coordination activities with partners, a focus group with traditional healers, and supervision at the Pluto checkpoint were also conducted.

North Kivu: Provision of IPC kits in 8 health centers in Beni by IMC (Boikene Health Center, Butsili Health Center, Bundji Health Center, Jades Medical Center, Tuungane Health Center, Mabakanga Health Center, Gracia Medical Center and Carle Becker Hospital); 155 service providers briefed, including 18 in Musienene, 4 in Kyondo, 57 in Butembo and 76 in Oïcha; Advocacy with 39 presidents of taxi driver associations and transport agencies in the city of Butembo for their involvement in raising awareness in their communities about the risks and respect for Ebola prevention measures in the Butembo health zone; Advocacy with 39 presidents of taxi driver associations and transport agencies in the city of Butembo for their involvement in raising awareness in their communities about the risks and respect for Ebola prevention measures in the Kalunguta health zone; Mass communication on prevention against Ebola virus disease and the raising of alerts in favor of 33 women, during the CPS in the Aloya AS in the Mabalako ZS; Two community dialogue sessions with 58 chiefs and community leaders on commitment to respecting barrier measures and strategies to combat the spread of Ebola virus disease, in the Katiri AS in the Mabalako ZS.

4.8. Logistics

Ituri

- Mobility ensured for the various pillars of the response; 25 vehicles and four (4) ambulances were made available. Supplies were also delivered to the pillars and beneficiary structures, including the CREC, the PCI, the CTEs and the health zones.

North Kivu:

- Delivery of IPC supplies to the EDS pillar (100 Tyvek coveralls, 50 bottles of hand sanitizer, 10 boxes of 50 masks, 100 face shields, 1 bag of OMO soap, 10 reusable aprons, 20 boxes of examination gloves, 5 gowns); Delivery of supplies
- EDS at the Mabalako Health Zone, including 15 adult body bags and 54 500ml bottles of hydroalcoholic gel and 5 child body bags.

4.9. Security

- Continued strike by service providers in the health zones of Bunia and Rwampara in Ituri.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Community awareness-raising on the prevention of sexual exploitation and abuse continued in the health zones of Bunia and Rwampara. A total of 415 people were reached (244 women/girls and 171 men/boys), including four (4) people living with disabilities.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge No.	Impact	Actions required
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1	Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, strengthen team security, and increase the number of EDS teams.
2	Insufficient capacity of CTEs and saturation in North Kivu (137.0%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy additional temporary structures, and improve laboratory turnaround times.
3	Contact tracking performance not optimal (80.0% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4	Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5	confirmed cases not yet ventilated by health zone (17 cases)	Major gaps in the reconstruction of transmission chains	Conduct a data reconciliation, prune any possible duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.) Active field verification
6	Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (112 cases) HCW (including 35 deaths, case fatality rate 31.2%)	Urgent supply, strengthening of IPC training
8	Stock of samples not yet analyzed (backlog) at the laboratory North Kivu and late diagnosis	Delayed confirmation, high lethality (56.8%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities, improve the flow of laboratory data
9	Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10	Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, and make provincial COUSPs operational.
11.	Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12	Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13	Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14.	High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

6. PHOTOS OF FIELD ACTIVITIES



Closing of the training for midwives and nurses in the Bambu health zone

**POUR TOUTE INFORMATION
SUPPLÉMENTAIRE, VEUILLEZ CONTACTER**

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Avec l'appui technique de l'Organisation Mondiale
de la Santé (OMS), de Africa CDC et Bluesquare

